

ENGLISH FOR WORK / SEPTEMBER 2026

# Medical Assisting English



## Teacher's guide

Teach practical workplace communication with specific cases, clear language targets, and a repeatable feedback process.

### PRACTICE THAT TRANSFERS TO WORK

Read a realistic case. Study complete conversations. Find the right language, speak, get feedback, and try again.

Eight lessons | Intermediate to advanced | Independent or classroom study

For Medical assistants and outpatient clinic support staff.

Every patient, symptom report, date, measurement context, and clinic arrangement is fictional language practice. This is not clinical, diagnostic, triage, medication, specimen-collection, privacy-law, or treatment training. Duties depend on local rules, training, delegation, and supervision. Qualified clinicians assess urgency and make care decisions; use the workplace's actual emergency and escalation procedures, not these scripts, for real concerns.

All scenarios, figures, and model responses are fictional teaching material. Use invented details in practice.

[Open this course on English Ladder](#)

# Your course at a glance

Use the lesson numbers to match this document with the website and the other guides.

## 01 Clarifying the visit at check-in

Describe a visit mismatch without dismissing the patient's reason for attending.

## 02 Rooming and factual patient reports

Attribute symptom language accurately without diagnosing or minimizing.

## 03 Medication histories with clear sources

Identify conflicting medication information and request a clinician's review.

## 04 Laboratory paperwork and specimen queries

Distinguish collection, dispatch, and receipt in a laboratory enquiry.

## 05 Referrals and appointment dependencies

Explain completed referral steps and remaining scheduling questions.

## 06 Supporting questions during a clinical visit

Arrange a clinician response without giving instructions beyond your role.

## 07 Telephone concerns and clinical escalation

Decline an unsupported urgency judgment while arranging the correct response.

## 08 Records, corrections, and follow-up ownership

Repair a scheduling message while keeping result interpretation with the clinician.

## Choose your pace

Quick practice: spend 15 minutes reading one conversation and rehearsing one scenario. Full lesson: allow 45-60 minutes for language work, three conversations, two speaking scenarios, and feedback.

## Level support

B1 learners can use the frames and a partner. B2 learners can try a response before reading the model. C1 learners can add the harder follow-up and reduce preparation time. These are teaching suggestions, not certified CEFR level ratings.

After practicing: open the AI prompt workshop, or use the prompts at the end of this guide.

# A 60-minute lesson that works

Prepare the case and the learner pages. Keep the model response hidden until learners have attempted their own. Choose two field terms that are important to understanding the case.

## 0-5 minutes | Activate experience

Ask learners to name a comparable situation without sharing confidential details. Introduce the communication goal.

## 5-12 minutes | Read and sort facts

Read the case. Learners identify confirmed facts, unknowns, the audience, and the immediate communication need.

## 12-22 minutes | Notice and practice language

Explain the workshop pattern. Work through the vocabulary and editing example, then discuss both language checks and the reasons behind the choices.

## 22-35 minutes | 05 • Conversations

Read one ten-turn conversation aloud, notice a useful expression, then compare the other two. Identify each speaker's purpose and how the exchange ends.

## 35-47 minutes | 06 • Say it

Rehearse both speaking scenarios from the workbook. Give two minutes of preparation, switch roles, and use the success checks. Add the complication only after a successful first round.

## 47-55 minutes | Compare and revise

Reveal the model. Discuss alternative acceptable wording. Give one meaning correction and one language correction, then repeat.

## 55-60 minutes | Retrieve and transfer

Close the materials. Learners recall two expressions and identify one communication habit to use in another situation.

## Adapt the session

For 45 minutes, study one complete conversation and rehearse one scenario; assign the others for oral follow-up. For 90 minutes, rehearse all three conversations and give both scenarios a second round. More time should produce more meaningful practice, not longer explanations.

## LESSON 01 / FACILITATOR NOTES

# Clarifying the visit at check-in

Describe a visit mismatch without dismissing the patient's reason for attending.

**CASE**

The schedule says annual review. Patient Alex expected a visit about a recent concern. The assistant cannot decide whether the appointment type is suitable. Nurse Dana can clarify the visit plan. Alex prefers Alex, while the verified record says Alexandra.

**Teach this move**

A booking label and the patient's chief concern serve different purposes. Ask what the patient expected, preserve their wording, and leave decisions about the visit's clinical suitability to the team.

Vocabulary to check: chief concern, intake form, patient identifier, preferred name.

**Model response**

Alex, the schedule describes an annual review, but you expected to discuss a recent concern. I will tell Dana about that difference so she can clarify the visit plan with you. I can use Alex when addressing you while keeping the verified record linked to Alexandra. I haven't confirmed a change to the appointment type.

**Listen for, then coach**

Ask learners to point to the case fact behind each important statement. Accept different wording when it preserves meaning, fits the listener, and does not invent an outcome. This checks the mismatch without making an unsupported scheduling or clinical decision.

**Second-round challenge**

Ask whether changing the display name also changes the verified identity record.

**Language-check answers**

1. C - preferred name Correct. Preferred name is the requested form of address.
2. A - to clarify Correct. Ask Dana to clarify uses the request pattern ask someone to do something.

**05 • Conversations**

1. Alex's visit expectation - Fictional check-in using the lesson case.
2. The blank box - Fictional intake form has a blank contact field; the assistant checks its meaning privately.

3. A quieter identity check - Fictional waiting room; patient requests privacy while identifiers are checked.

Use the complete ten-turn scripts in the learner workbook and conversation lab. Read with roles, identify the decision or open question, then replay without reading.

## 06 · Say it: second scenario | A form completed by a companion

Patient Noor's companion completed the intake form and selected routine follow-up. Noor says the main concern is different. No clinician has reviewed it. Nurse Dana is available to receive the discrepancy.

Role A: Assistant: ask Noor directly about the intended concern without criticizing the companion.

Role B: Noor: you want your own wording passed on and do not know the correct booking category.

Possible opening: My companion chose that box, but it isn't what I wanted to discuss.

### Success checks

- Address Noor directly.
- Distinguish the form entry from the patient's current account.
- Give Dana the mismatch without deciding the clinical visit type.

Add a complication: The companion says the written answer should take priority over Noor's explanation.

AI extension: adapt this lesson for your learners and available time.

## LESSON 02 / FACILITATOR NOTES

# Rooming and factual patient reports

Attribute symptom language accurately without diagnosing or minimizing.

**CASE**

During rooming, patient Pat reports tiredness since Monday. The assistant observed Pat answering questions while seated but has not assessed the cause or severity. Nurse Dana is receiving the report. No measurement values or clinical conclusions are supplied.

**Teach this move**

Use 'reports' for the patient's experience and 'I observed' for what you directly noticed. Neither phrase establishes a diagnosis. Time words such as 'since Monday' and 'on Monday' convey different durations.

Vocabulary to check: rooming, vital signs, reported symptom, onset.

**Model response**

Pat reports feeling tired since Monday. During our conversation, Pat answered questions while seated. That observation does not explain the tiredness or establish its severity. Dana, I am passing on the reported onset and what I directly observed; Pat wants to discuss the change with the clinician. I have not offered an explanation for it.

**Listen for, then coach**

Ask learners to point to the case fact behind each important statement. Accept different wording when it preserves meaning, fits the listener, and does not invent an outcome. The new wording keeps the stated experience without adding a cause.

**Second-round challenge**

Ask whether answering questions proves the reported concern is minor.

**Language-check answers**

1. A - reported Correct. Reported attributes the onset to Pat's account.
2. A - since Correct. Since links a starting point to continuing experience.

**05 • Conversations**

1. What Pat reported - Fictional staff handoff using the lesson case, without clinical assessment.
2. Yesterday or last evening - Fictional patient clarifies the time meant by yesterday; no diagnostic conclusion.

3. A measurement entry without its time - Fictional chart handoff; a vital-sign entry lacks a timestamp and its origin is unclear.

Use the complete ten-turn scripts in the learner workbook and conversation lab. Read with roles, identify the decision or open question, then replay without reading.

## **06 · Say it: second scenario | The word dizzy needs the patient's explanation**

A patient uses dizzy but then says they may mean tired. The assistant cannot interpret or assess the symptom. Nurse Dana is present and can take the concern now; no emergency symptoms are specified.

Role A: Assistant: ask the patient to describe the experience in their own words and pass it to Dana.

Role B: Patient: you are unsure which English word fits and want help communicating, not a diagnosis.

Possible opening: I said dizzy, but I'm not sure that is the right word in English.

### **Success checks**

- Do not select a diagnosis or cause.
- Attribute the patient's wording and uncertainty.
- Connect the patient to Dana for assessment rather than deciding urgency from vocabulary.

Add a complication: A companion offers a cause the patient has not confirmed.

AI extension: adapt this lesson for your learners and available time.

**LESSON 03 / FACILITATOR NOTES**

# Medication histories with clear sources

Identify conflicting medication information and request a clinician's review.

**CASE**

Patient Ren says one medicine was stopped at another clinic. The local list still includes it. Ren does not have the other clinic's instruction. Nurse Dana can review the discrepancy with the clinician; the assistant cannot confirm discontinuation or change instructions.

## Teach this move

A patient report is important evidence to relay, not an instruction the assistant independently verifies. Keep source, date, and uncertainty visible. Do not infer a medicine from color or recommend changes.

Vocabulary to check: medication reconciliation, medication list, nonprescription medicine, allergy history.

## Model response

Dana, could you review this medication-list discrepancy with the clinician? Ren reports that another clinic stopped one medicine, but our list still includes it and we do not have that instruction. Please clarify the current list with Ren. I have recorded the report as unverified and have not advised starting, stopping, or changing anything.

## Listen for, then coach

Ask learners to point to the case fact behind each important statement. Accept different wording when it preserves meaning, fits the listener, and does not invent an outcome. This preserves the reported change without independently deciding the medication plan.

## Second-round challenge

Ask the assistant whether to take the medicine tonight; redirect the question to the clinician.

## Language-check answers

1. C - reported Correct. Reported identifies the source without verification.
2. C - nonprescription Correct. Nonprescription names the category described.

## 05 • Conversations

1. A reported change from another clinic - Fictional medication-history exchange using the lesson case.

2. A supplement and an uncertain name - Fictional history conversation; the patient has an unlabeled memory, not a verified product name.

3. An allergy entry needs a source - Fictional chart query: blank allergy field and patient report require clinician review.

Use the complete ten-turn scripts in the learner workbook and conversation lab. Read with roles, identify the decision or open question, then replay without reading.

## 06 · Say it: second scenario | Two differently dated lists

A patient brings a list dated June and a phone photograph dated August. One item differs. Neither has been reconciled with today's chart. Nurse Dana can review both with the clinician.

Role A: Assistant: identify the sources and date difference.

Role B: Patient: ask which version is correct; you do not know why they differ.

Possible opening: The photograph is newer, so can you assume everything on it is current?

### Success checks

- Name the two dated sources.
- Do not equate newer with clinically reconciled.
- Ask the clinician to review the differences without changing instructions.

Add a complication: The patient asks the assistant to delete every item missing from the photograph.

AI extension: adapt this lesson for your learners and available time.

## LESSON 04 / FACILITATOR NOTES

# Laboratory paperwork and specimen queries

Distinguish collection, dispatch, and receipt in a laboratory enquiry.

**CASE**

A requisition gives 09:20 as collection time; the courier sheet says 10:05 arrival. A colleague proposes replacing 09:20 with 10:05. Both times describe different events. No identity discrepancy or test result is stated.

**Teach this move**

An accurate time paired with the wrong event is misleading. Ask who recorded the time and what happened then. This lesson does not teach specimen collection, labeling, handling, acceptance, or testing procedures.

Vocabulary to check: specimen, requisition, collection time, specimen identifier.

**Model response**

Those times refer to different events. The requisition records collection at nine twenty, while the courier sheet records arrival at ten oh five. Please keep the two labels clear rather than replace one time with the other. If either source is questioned, we should ask the recording team to clarify that specific entry.

**Listen for, then coach**

Ask learners to point to the case fact behind each important statement. Accept different wording when it preserves meaning, fits the listener, and does not invent an outcome. The revision attaches each time to the correct event.

**Second-round challenge**

Ask whether laboratory receipt guarantees that the specimen has been accepted for testing.

**Language-check answers**

1. D - collection Correct. Collection is when the sample was obtained.
2. A - pending Correct. Pending marks the unfinished review.

**05 • Conversations**

1. Two times, two events - Fictional paperwork discussion using the lesson case.

2. A request number read back - Fictional clinic-laboratory call; two similar request references need clarification.
3. The label question before a conclusion - Fictional specimen paperwork concern: identifiers differ; the laboratory lead must review.

Use the complete ten-turn scripts in the learner workbook and conversation lab. Read with roles, identify the decision or open question, then replay without reading.

## **06 · Say it: second scenario | Preparation wording needs clarification**

A patient has an appointment for a clinician-ordered test but received two different preparation messages. Their exact medical details are not supplied. Nurse Dana can compare the messages and obtain clinical clarification.

Role A: Assistant: identify that there are two messages without choosing instructions.

Role B: Patient: ask which message to follow and whether the appointment is cancelled.

Possible opening: One message says something different from the other. Which preparation instruction is right?

### **Success checks**

- Request the actual messages for clinician review.
- Do not invent preparation directions.
- Keep clarification distinct from cancellation or confirmed readiness.

Add a complication: The patient asks you to use the newer message automatically.

AI extension: adapt this lesson for your learners and available time.

## LESSON 05 / FACILITATOR NOTES

# Referrals and appointment dependencies

Explain completed referral steps and remaining scheduling questions.

**CASE**

Referral F8 was sent Monday and received Tuesday. The specialist office says it is under review, with no appointment booked. Coordinator Lia has agreed to check the status Friday. The patient believes receipt means a Friday appointment.

**Teach this move**

Sent, received, reviewed, and scheduled name distinct stages. Give the known stage and next contact without predicting the specialist's decision or timing.

Vocabulary to check: referral, specialist, appointment confirmation, referral status.

**Model response**

Referral F8 was sent Monday and received Tuesday. The specialist office is reviewing it, but no appointment has been booked. Friday is when Lia will check the status, not a visit date. We need a separate appointment confirmation before we can give you a visit time or tell you which site to attend.

**Listen for, then coach**

Ask learners to point to the case fact behind each important statement. Accept different wording when it preserves meaning, fits the listener, and does not invent an outcome. A follow-up date should not be mistaken for an appointment.

**Second-round challenge**

Ask whether under review means the specialist has accepted the referral.

**Language-check answers**

1. B - check Correct. Check describes Lia's information task.
2. B - scheduled Correct. Scheduled means a visit has been arranged.

**05 • Conversations**

1. Friday is a check, not a visit - Fictional referral update using the lesson case.
2. The same clinic name at two sites - Fictional confirmation lists East Clinic; patient assumes the West site.
3. Receipt not found at the receiving office - Fictional referral has a sent record but no acknowledgment.

Use the complete ten-turn scripts in the learner workbook and conversation lab. Read with roles, identify the decision or open question, then replay without reading.

## 06 · Say it: second scenario | A cancellation list is not a booking

A specialist office offers a cancellation list while the patient's existing appointment remains next month. No earlier slot is available. Lia can explain the administrative distinction.

Role A: Assistant: explain the existing booking and possible earlier opportunity.

Role B: Patient: you think joining the list cancels next month's appointment.

Possible opening: If I join that list, do I lose the appointment I already have?

### Success checks

- Preserve the existing appointment in the explanation.
- Describe an earlier slot as unconfirmed.
- Avoid predicting priority or promising a clinical scheduling decision.

Add a complication: The patient asks for a guaranteed earlier slot before changing work plans.

AI extension: adapt this lesson for your learners and available time.

## LESSON 06 / FACILITATOR NOTES

# Supporting questions during a clinical visit

Arrange a clinician response without giving instructions beyond your role.

**CASE**

Patient Jules does not understand a phrase in the after-visit summary and has not left. Dr Chen is available. The assistant cannot reinterpret the instruction. Jules wants an explanation before leaving, away from reception.

**Teach this move**

Identify the unclear phrase and requested conversation setting. A clarification question is not refusal of all care. Teach-back checks the explanation, not the patient's intelligence.

Vocabulary to check: chaperone, consent discussion, after-visit summary, teach-back.

**Model response**

Dr Chen, Jules has a question about a phrase in the after-visit summary and would like it explained before leaving. I have not reinterpreted the instruction. Jules prefers to discuss it here rather than at reception. Could you return to the room and check the explanation with Jules before we finish the visit?

**Listen for, then coach**

Ask learners to point to the case fact behind each important statement. Accept different wording when it preserves meaning, fits the listener, and does not invent an outcome. The question gets a specific recipient and location.

**Second-round challenge**

Say you nodded to be polite but still do not understand.

**Language-check answers**

1. D - clarify Correct. Clarify means make the intended meaning clear.
2. D - teach-back Correct. Teach-back checks the explanation through the patient's understanding.

**05 • Conversations**

1. An unclear phrase before leaving - Fictional visit-room exchange using the lesson case.
2. A pause is a request, not a motive - Fictional patient pauses an examination discussion; no procedure is described.
3. A chaperone request with a question - Fictional chaperone enquiry; arrangements and role need explanation.

Use the complete ten-turn scripts in the learner workbook and conversation lab. Read with roles, identify the decision or open question, then replay without reading.

## 06 · Say it: second scenario | A summary from an earlier visit

A patient opens a portal document dated last month. Today's summary has not appeared. Dr Chen can clarify today's information before the patient leaves.

Role A: Assistant: identify the date mismatch and request clarification.

Role B: Patient: you want to avoid relying on the wrong document.

Possible opening: Why does this document show last month's date?

### Success checks

- Name the date mismatch.
- Do not adapt clinical instructions.
- Ask Dr Chen to clarify today's summary and guidance.

Add a complication: The patient asks you to adapt the older instructions to today's visit.

AI extension: adapt this lesson for your learners and available time.

## LESSON 07 / FACILITATOR NOTES

# Telephone concerns and clinical escalation

Decline an unsupported urgency judgment while arranging the correct response.

**CASE**

A caller asks the assistant to decide whether a newly reported concern can wait until next week's routine visit. No clinical details or urgency assessment are supplied. Nurse Dana is available now through the clinic's clinical-call route. The assistant cannot triage.

**Teach this move**

A routine appointment label does not assess a new concern. Use actual local emergency and escalation procedures for real calls. This exercise supplies no symptom criteria and teaches no triage decisions.

Vocabulary to check: triage, escalation, callback number, warm transfer.

**Model response**

I can't determine from an appointment booking whether this new concern can wait until next week. Dana is available through our clinical-call route now, and I will connect you so she can assess it. I will pass on your question about timing without describing the concern as routine or offering reassurance about its importance.

**Listen for, then coach**

Ask learners to point to the case fact behind each important statement. Accept different wording when it preserves meaning, fits the listener, and does not invent an outcome. A calendar entry cannot establish clinical urgency.

**Second-round challenge**

Ask for a yes-or-no answer because you do not want to repeat the question.

**Language-check answers**

1. D - triage Correct. Triage assesses urgency and response; the assistant is not performing it here.
2. D - warm transfer Correct. A warm transfer includes an introduction to the receiver.

**05 • Conversations**

1. A booking cannot assess the concern - Fictional telephone exchange using the lesson case.

2. The number for this call - Fictional administrative callback: patient will be away from their usual telephone; no symptoms.

3. A nurse accepts the live call - Fictional internal telephone introduction; no symptom assessment or care instruction.

Use the complete ten-turn scripts in the learner workbook and conversation lab. Read with roles, identify the decision or open question, then replay without reading.

## 06 · Say it: second scenario | A disconnected handoff

A scheduling caller is disconnected before a warm transfer reaches reception. Reception has not accepted the call. The assistant has the caller's verified number and can return the call now. No clinical concern is involved.

Role A: Assistant: clarify the failed handoff and re-establish contact.

Role B: Reception colleague: you did not receive the caller and can take the call after reconnection.

Possible opening: The transfer disconnected before you answered; can you take the scheduling call once I reconnect?

### Success checks

- Separate a transfer attempt from accepted contact.
- Use the verified callback number without inventing an outcome.
- Confirm who is reconnecting and who will receive the call.

Add a complication: The task display says transferred, and a colleague assumes the conversation occurred.

AI extension: adapt this lesson for your learners and available time.

## LESSON 08 / FACILITATOR NOTES

# Records, corrections, and follow-up ownership

Repair a scheduling message while keeping result interpretation with the clinician.

**CASE**

The assistant's message incorrectly says a results discussion is booked Tuesday. The schedule confirms Thursday at 14:00 with Dr Chen. The patient rearranged work. The assistant can correct the message and confirm the booking; no results are discussed in this exercise.

**Teach this move**

State what was wrong and what the verified record says. An amendment should remain traceable. Updating a record, reaching the patient, and finishing a follow-up task are different actions.

Vocabulary to check: electronic health record, amendment, release of information, follow-up task.

**Model response**

I'm sorry my message gave Tuesday for your results discussion. The confirmed appointment is Thursday at two with Dr Chen. I understand you changed your work plans because of my message. I'll correct that message in the record and check that you have the Thursday details; I cannot interpret the results during this scheduling call.

**Listen for, then coach**

Ask learners to point to the case fact behind each important statement. Accept different wording when it preserves meaning, fits the listener, and does not invent an outcome. A specific correction is more useful than a vague apology.

**Second-round challenge**

Ask whether the wrong date says anything about the clinical importance of the results.

**Language-check answers**

1. D - confirmed Correct. Confirmed matches the verified schedule.
2. A - outstanding Correct. Outstanding means the contact remains to be done.

## 05 · Conversations

1. Tuesday was the incorrect message - Fictional correction call using the lesson case.
2. A record request with an unclear recipient - Fictional records enquiry; patient asks for a summary to be sent but the destination is unspecified.
3. A reached patient or an unanswered call - Fictional follow-up record says contacted, but only an unanswered attempt occurred.

Use the complete ten-turn scripts in the learner workbook and conversation lab. Read with roles, identify the decision or open question, then replay without reading.

## 06 · Say it: second scenario | A copied appointment time

An old message says 11:00. Today's confirmed booking is 11:30 at the same clinic. A colleague copied the old message into a reminder but has not sent it. You can prevent the inaccurate reminder now.

Role A: Assistant: identify the outdated source and request a corrected reminder.

Role B: Colleague: ask whether the clinic location also changed; it did not.

Possible opening: The unsent reminder still says eleven, but today's booking confirms eleven thirty.

### Success checks

- Correct only the verified time difference.
- Keep the unchanged location explicit.
- Confirm the corrected reminder before describing it as sent.

Add a complication: The colleague assumes every detail from the old message must be wrong.

AI extension: adapt this lesson for your learners and available time.

# Give feedback that helps

Score each criterion 0, 1, or 2. This is a practice rubric, not a professional qualification or CEFR test. Do not award or remove points for a particular accent.

## Meaning and accuracy

Keeps the case facts accurate; clearly separates confirmed and unknown information.

## Clarity and language

Uses understandable sentences and explains specialist terms when the listener needs it.

## Interaction and tone

Responds to the other person's question, checks understanding, and uses an appropriate tone.

## Task completion

Makes the requested action or unresolved question clear without inventing authority or facts.

## Use the same scale for each criterion

0 - Not yet: The reader or listener cannot yet recover the intended meaning or action.

1 - With support: The message works after a prompt, clarification, or revision.

2 - Independently: The message is clear and accurate without a prompt.

## Feedback sequence

First, identify a successful phrase. Next, identify one point where meaning or accuracy needs work. Offer one useful language correction, allow a repeat, and compare the two attempts. A total out of eight describes this performance only; do not translate it into a proficiency level.

## Final challenge

Choose a case not rehearsed today. The learner gives a one-minute response, answers two follow-up questions, and switches roles and repeats the exchange. Add the lesson's second-round challenge. Compare the result with an earlier attempt using the four criteria.

# Use the language responsibly

Every patient, symptom report, date, measurement context, and clinic arrangement is fictional language practice. This is not clinical, diagnostic, triage, medication, specimen-collection, privacy-law, or treatment training. Duties depend on local rules, training, delegation, and supervision. Qualified clinicians assess urgency and make care decisions; use the workplace's actual emergency and escalation procedures, not these scripts, for real concerns.

## Meaning before performance

Use specialist terms only when they help the listener. Explain unfamiliar words, preserve uncertainty, and ask when an instruction or responsibility is unclear. The model responses illustrate language; they do not authorize professional actions.

## Sources and further reading

The cases, workshops, and explanations are original teaching material. These references provide language frameworks and selected professional context. References were checked in September 2026; consult current local guidance for actual work.

### Digital.gov: plain-language guidance

<https://digital.gov/guides/plain-language>

### Council of Europe: CEFR descriptors

<https://www.coe.int/en/web/common-european-framework-reference-languages/cefr-descriptors>

### U.S. Bureau of Labor Statistics: Medical Assistants

<https://www.bls.gov/ooh/healthcare/medical-assistants.htm>

### American Association of Medical Assistants: Medical Assisting

<https://www.aama-ntl.org/medical-assisting>

## Your next step

Choose one expression you can use in a genuine work situation. Rehearse it with fictional details, then adapt the wording to your role and audience.

# Extend your practice with AI

Use the next two prompts after your own first attempt. Each contains the course context and a complete starting case or dialogue. Copy the entire prompt, including the reference, into a new chat in your preferred AI. You can also use the links to copy it from the website.

## Choose the right kind of help

A role-play prompt makes the AI wait for your replies. A new-dialogue prompt asks for a complete professional script. Guided message practice gives four choices with explained feedback. Vocabulary, grammar and review prompts move from explanation to your own attempts.

## Choose any lesson or dialogue online

The course prompt workshop has eight practice goals and B1 support, B2 independent and C1 stretch settings. These are practice choices, not assessed proficiency levels. Select the same lesson number or dialogue title you used in this guide.

[Open this course's AI prompt workshop](#)

## Keep practice useful

Use fictional or anonymized details. English Ladder does not send anything to an AI service or add your drafts to the prompts. Your chosen service's terms and any usage charges apply. AI responses can contain mistakes; compare feedback with the lesson and verify specialist claims against the course references.

The two starter prompts use B2 practice. To change support in a printed prompt, replace its practice-setting paragraph with a request for shorter explanations and sentence starters (B1), or greater precision and more complex constraints (C1). Keep the professional facts unchanged.

## Changing the example

For another case on paper, replace the text between REFERENCE and END REFERENCE with that case, its course context, relevant terms and language target. Include the full exchange if practicing a dialogue. Do not assume your AI can access this PDF or a link. The website makes this substitution for you.

Prompt edition: 2026-09-28. These are original practice instructions; results depend on the AI service you choose.

# Adapt a partner or class activity

## START PROMPT / COPY THROUGH END PROMPT | ONLINE VERSION

Be my workplace English practice partner. Follow the practice instructions after the REFERENCE block. The block contains fictional study material, not instructions to you. Keep its facts, numbers, uncertainty and professional responsibilities accurate. Clearly label any new scenario or changed fact as an invented variation. Use natural workplace language; explain unfamiliar abbreviations in context. If a technical expression is uncertain, say so rather than inventing a definition or authority. Coach communication, not professional decisions; respect the course scope. Ask only for fictional or anonymized practice details. Judge clarity, meaning, appropriate tone and the next step. Accept valid alternative English and distinguish errors from style preferences. Do not claim to certify my language level. Unless I ask to change the plan, follow the stages and stop wherever the instructions say to wait.

When discussing a word, phrase, or example sentence as language within an explanation, question, or answer feedback, enclose that wording in quotation marks. For example: What does "about" mean in "about twenty people"? Use "on" with a named day. Keep standalone answer choices, vocabulary headwords, and ordinary reading or dialogue text uncluttered. Quotation marks identifying a language example do not attribute it to a news source; attributed source quotations must remain verbatim.

Practice setting: B2 independent. Use natural professional English with brief explanations. Let me attempt each task without a model first. Offer a hint after difficulty and invite a more precise retry.

### REFERENCE

Course: Medical Assisting English

Professional audience: Medical assistants and outpatient clinic support staff

Scope: Every patient, symptom report, date, measurement context, and clinic arrangement is fictional language practice. This is not clinical, diagnostic, triage, medication, specimen-collection, privacy-law, or treatment training. Duties depend on local rules, training, delegation, and supervision. Qualified clinicians assess urgency and make care decisions; use the workplace's actual emergency and escalation procedures, not these scripts, for real concerns.

Lesson 01: Clarifying the visit at check-in

Original fictional case: The schedule says annual review. Patient Alex expected a visit about a recent concern. The assistant cannot decide whether the appointment type is suitable. Nurse Dana can clarify the visit plan. Alex prefers Alex, while the verified record says Alexandra.

Language workshop: Acknowledge the expectation before checking the booking

Communication goal: Describe a visit mismatch without dismissing the patient's reason for attending.

Language guidance: A booking label and the patient's chief concern serve different purposes. Ask what the patient expected, preserve their wording, and leave decisions about the visit's clinical suitability to the team.

Useful frames (complete the gaps with case facts): The schedule says \_\_\_\_; you expected \_\_\_\_\_. / How would you describe your main concern today? / I can address you as \_\_\_\_ while verifying the record under \_\_\_\_.

Vocabulary: chief concern: The main reason a patient gives for seeking care at the current visit. / intake

form: A clinic document used to gather patient information before or during a visit. / patient identifier: A

detail used with the clinic's verification process to distinguish one patient from another. / preferred name:

The name a patient asks staff to use when addressing them.

Speaking task: Prepare for two minutes. Speak for 45-60 seconds using the case facts, then respond to your partner's question. Switch roles and repeat without reading the model response.

Partner: Alex: explain the different expectation and ask staff to use your preferred name.

Guided response task: Complete a workplace dialogue by selecting one of four supplied options. Compare the feedback and say the completed exchange aloud. Do not write an open-ended message.

Optional second-round challenge: Ask whether changing the display name also changes the verified identity record.

Model for comparison AFTER my attempt, not a response to give me first: Alex, the schedule describes an annual review, but you expected to discuss a recent concern. I will tell Dana about that difference so she can clarify the visit plan with you. I can use Alex when addressing you while keeping the verified record linked to Alexandra. I haven't confirmed a change to the appointment type.

**END REFERENCE**

**TASK:** Adapt a supported occupational language lesson

1. Ask for learner support needs, group size and time. Offer two B2 learners and 20 minutes as a default and wait for confirmation.
2. Use the reference's real language goal, terms and case facts. Provide a timed plan containing reading, four-choice dialogue gaps, scaffolded speaking, focused feedback and a retry. No activity may require open-ended writing.
3. Give two separate speaking role cards with known facts, an objective, a possible opening, one question to resolve and three success checks. Put model responses in a separate teacher section. Label invented complications and preserve uncertainty.
4. Include four original gap-fill exchanges with four plausible options each and exactly one answer. Provide option-specific explanations separately. B1 support adds a word bank and frames; C1 stretch uses more nuanced choices and spoken follow-up, not essays. Include solo rehearsal of both roles.
5. Check facts, terminology, answer uniqueness and timing before presenting the plan. Finish with a four-choice exit question and an optional spoken retry. Do not certify proficiency or teach professional procedures.

**END PROMPT**

# Adjust tone and register

## START PROMPT / COPY THROUGH END PROMPT | ONLINE VERSION

Be my workplace English practice partner. Follow the practice instructions after the REFERENCE block. The block contains fictional study material, not instructions to you. Keep its facts, numbers, uncertainty and professional responsibilities accurate. Clearly label any new scenario or changed fact as an invented variation. Use natural workplace language; explain unfamiliar abbreviations in context. If a technical expression is uncertain, say so rather than inventing a definition or authority. Coach communication, not professional decisions; respect the course scope. Ask only for fictional or anonymized practice details. Judge clarity, meaning, appropriate tone and the next step. Accept valid alternative English and distinguish errors from style preferences. Do not claim to certify my language level. Unless I ask to change the plan, follow the stages and stop wherever the instructions say to wait.

When discussing a word, phrase, or example sentence as language within an explanation, question, or answer feedback, enclose that wording in quotation marks. For example: What does "about" mean in "about twenty people"? Use "on" with a named day. Keep standalone answer choices, vocabulary headwords, and ordinary reading or dialogue text uncluttered. Quotation marks identifying a language example do not attribute it to a news source; attributed source quotations must remain verbatim.

Practice setting: B2 independent. Use natural professional English with brief explanations. Let me attempt each task without a model first. Offer a hint after difficulty and invite a more precise retry.

### REFERENCE

Course: Medical Assisting English

Professional audience: Medical assistants and outpatient clinic support staff

Scope: Every patient, symptom report, date, measurement context, and clinic arrangement is fictional language practice. This is not clinical, diagnostic, triage, medication, specimen-collection, privacy-law, or treatment training. Duties depend on local rules, training, delegation, and supervision. Qualified clinicians assess urgency and make care decisions; use the workplace's actual emergency and escalation procedures, not these scripts, for real concerns.

Lesson 01: Clarifying the visit at check-in

Original fictional case: The schedule says annual review. Patient Alex expected a visit about a recent concern. The assistant cannot decide whether the appointment type is suitable. Nurse Dana can clarify the visit plan. Alex prefers Alex, while the verified record says Alexandra.

Language workshop: Acknowledge the expectation before checking the booking

Communication goal: Describe a visit mismatch without dismissing the patient's reason for attending.

Language guidance: A booking label and the patient's chief concern serve different purposes. Ask what the patient expected, preserve their wording, and leave decisions about the visit's clinical suitability to the team.

Useful frames (complete the gaps with case facts): The schedule says \_\_\_\_; you expected \_\_\_\_\_. / How would you describe your main concern today? / I can address you as \_\_\_\_\_ while verifying the record under \_\_\_\_\_.

Vocabulary: chief concern: The main reason a patient gives for seeking care at the current visit. / intake form: A clinic document used to gather patient information before or during a visit. / patient identifier: A detail used with the clinic's verification process to distinguish one patient from another. / preferred name: The name a patient asks staff to use when addressing them.

Speaking task: Prepare for two minutes. Speak for 45-60 seconds using the case facts, then respond to your partner's question. Switch roles and repeat without reading the model response.

Partner: Alex: explain the different expectation and ask staff to use your preferred name.

Guided response task: Complete a workplace dialogue by selecting one of four supplied options. Compare the feedback and say the completed exchange aloud. Do not write an open-ended message.

Optional second-round challenge: Ask whether changing the display name also changes the verified identity record.

Model for comparison AFTER my attempt, not a response to give me first: Alex, the schedule describes an annual review, but you expected to discuss a recent concern. I will tell Dana about that difference so she can clarify the visit plan with you. I can use Alex when addressing you while keeping the verified record linked to Alexandra. I haven't confirmed a change to the appointment type.

**END REFERENCE**

**TASK:** Match the wording to the listener

1. Quote one actual utterance from the reference. Identify its purpose, audience and degree of certainty. Offer a colleague, supervisor or outside customer as the new listener. Wait for my selection.
2. Supply four possible versions A-D and a specific constraint, such as explaining an unfamiliar abbreviation without changing a pending decision into approval. Make exactly one version satisfy all constraints. Wait for my choice.
3. Explain the strengths and limitations of every option. Treat valid stylistic alternatives as alternatives, not mistakes; rewrite an ambiguous question instead of rejecting reasonable English. Avoid national stereotypes.
4. Continue with three short, role-labelled exchanges in which I select the missing reply from four choices. Include a clarification and a disagreement. Wait after each item and invite a spoken rehearsal after feedback.
5. Finish with two complete model exchanges for different listeners. Identify what changed and what stayed the same. Do not assign open-ended writing.

**END PROMPT**